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Educational Specialist, 2020-current. 97.
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Divining the Self: Applying Tarot as a Projective Technique in Counseling

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A research project submitted to the Graduate Faculty of

JAMES MADISON UNIVERSITY

In

Partial Fulfillment of the Requirements

for the degree of

Educational Specialist

Department of Graduate Psychology

May 2024

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Dedication Page

To Bennett: for gifting me my first set of tarot cards, and for guiding me to believe in a spirituality that is intuitive and kind.

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Abstract

Projective techniques comprise a controversial category of interventions in therapy, but when utilized appropriately, can inspire valuable progress in the counseling process. This paper first explores the clinical benefits of projectives and scholarly concerns regarding their use in a clinical setting, before providing a brief overview of the history and practice of tarot card reading. Tarot has infrequently been the focus of academic research as a projective technique, and is presented here as a secular medium to enrich the therapeutic work and relationship. If utilized from an expressive rather than a diagnostic approach, this paper argues for the application of tarot as a projective technique in counseling with an appropriate client base, and further advocates for additional investigation of the intervention's effectiveness in deepening the therapeutic process and enriching the therapeutic alliance.

Keywords: projective assessment, projective techniques, projective interventions, tarot in counseling

Introduction

For as long as projective techniques have been utilized in therapy, they have maintained a reputation of controversy and caution. Skeptics of the modality frequently cite the possible risks of clinician incompetence and inexperience, as well as psychometric limitations, as primary arguments for avoidance (Clark, 1995; Darley & Hagenah, 1955; Hahn & MacLean, 1955). However, with proper practice and mitigation of such risks, projective techniques hold the potential to inspire valuable and substantive progress in the counseling process (Clark, 1995; Moller et al., 2021). Broadly, projective techniques are an unstructured form of psychological assessment in which users are asked to respond to ambiguous stimuli (often visual or pictorial), and are believed to “project” their own unconscious and hidden emotions, personal histories, and internal conflicts onto the stimuli (Gibbs, Marchant, & Andersen, 2016). Rooted in psychoanalytic theory, projective tests originated in the early twentieth century with Swiss psychiatrist Hermann Rorschach, who developed the now ubiquitous Rorschach inkblot test (Hertz, 1986). Today, a number of projective assessments of varying styles and administration exist and offer alternative methods and tools for strengthening the therapeutic alliance and obtaining insight about individual clients (Clark, 1995; Wood & Pignatelli, 2019).

While traditionally believed to be oracular in nature rather than an example of deliberate personal projection, tarot reading is a centuries-old form of cartomancy in which practitioners are believed to gain insight into their own or another individual’s past, present, future, and overall spiritual path or identity. However, if one permits elimination of the belief in the prophetic ability of the cards or their reader, as well as a flexibility beyond traditional card interpretations, the practice of tarot reading may be considered an example of projective assessment not unlike some used formally and informally in counseling rooms today. By first providing a brief overview of

the history, application, and value of projective techniques in counseling, offering an introduction to and theoretical review of the practice of tarot, and finally proposing a sample manual for practice, this paper seeks to present tarot as a meaningful and compelling projective intervention in a therapeutic setting.

Projective Techniques in Counseling

In order for a psychological test or assessment to be considered projective, it is typically characterized by “relatively unstructured ambiguous stimuli”, and/or gives a description of the test to the user which is “a screen used to cover up the real purpose of the test, i.e. getting at personality dimensions” (Langland, 1960, p. 102). Thus, there are virtually unlimited options for the modality of a projective technique. With this in mind, however, an overview of the history of projective assessments shows that many of the most commonly used examples are visual in nature, and further require the user to respond to stimuli either verbally or through physical ability such as drawing, writing, or tactile manipulation of objects.

Origins and History of Projective Techniques

Into the early 20th century, psychometric approaches ruled the field of personality assessment in psychology, with paper-and-pencil tests emphasizing concentration on “single traits or dimensions” (Hertz, 1986, p. 397). However, as Hertz describes in her account of the development of the *Journal of Personality Assessment*, some researchers during this time chose to lean into the construction and use of less rigid instruments that attempted to view an individual’s personality as holistic. Their goals were to “elicit information as to the structure, the dynamics, and the functioning of the mental life of an individual” and “focus on the unity of the personality” (1986, p. 397). Notably, in 1921 Swiss psychiatrist and Jungian student Hermann Rorschach published the results of his experiments in which users verbally explained their visual

perceptions of 10 symmetrical inkblots. He then interpreted these responses through the lenses of personality and psychopathology. Hertz notes that while it is a matter of debate if Rorschach would have considered his own method to be projective, it paved the way for more intentionally ambiguous approaches to assessment (1986). Almost twenty years later, Murray and Morgan spearheaded the official projective movement with their Thematic Apperception Test (TAT), in which researchers examine users' personalities based on their impressions of pictures portraying vague social situations (Hertz, 1986). One year later in 1939, Frank published his classic piece "Projective Methods for the Study of Personality" in which he asserts that personality can be viewed as "the way an individual organizes and patterns life situations and effectively responds to them" (1939, p. 403). Further, in using projective methods, he argues, researchers are "evoking the very process of personality as it has been developed to that moment" and gaining insights into personality which quantitative measures "seem deliberately to reject" (1939, p. 408).

The arrival of the TAT in 1938 spurred the creation of new assessments, a trend that extended into the 1950s and included adaptations of the original TAT for children such as Blum's Blacky pictures test, which features the adventures and conflicts of a black dog; and the aptly named Children's Apperception Test, which displays animals in "existential or dangerous situations" (Lemov, 2011, p. 262). Other age groups were represented in their own targeted assessments, such as the Adolescent Fantasy Test and the Senior Apperception Test, and visual approaches to projection were generally emphasized during this era with techniques like the Draw-A-Person test and Make-A-Picture-Story test (Lemov, 2011, p. 262). Lemov asserts that World War II was the most influential catalyst for this flood of new scholarship and research in projectives (2011). During this time, she explains, many psychologists and anthropologists

joined the war effort through psychological assessment of U.S. military applicants and individuals from various cultures across the globe (Lemov, 2011). In fact, projective tests were designed not only for specific age groups, but for individuals of specific ethnic identities and situations, including African-Americans, Vietnamese, and Korean prisoners of war, for ethnographic purposes (Lemov, 2011).

Current Status of Projective Assessment in Counseling

In the second half of the 20th century, projective techniques fell out of favor in the psychological assessment world. Bellak concludes that this was due to the onset and universalization of drug therapy, as it supported the growth of community mental health and precipitated a more general decline in the interest in “psychodynamics, psychodiagnostics, as well as psychotherapy” (1992, p. 449). However, over the past few decades, the state of projective techniques in the counseling and psychology world has presented a paradox in terms of reputation versus praxis: as Anastasi mused, “projective techniques present a curious discrepancy between research and practice. When evaluated as psychometric instruments, the large majority make a poor showing. Yet their popularity in clinical use continues unabated” (1982, p. 564). Lilienfeld et al. confirm that the popularity of projective tests among many clinicians has ebbed, citing the impact of managed care as a potential factor (2000). Clark notes that training in projective assessment at the Master’s level has experienced a significant reduction, if not stopped altogether, based on surveys of programs (1995). This trend has continued, as a 2018 writing by Stedman et al. reports that a decrease in training emphasis for projectives has been seen across both graduate and internship training programs since the 1990s, alongside more firm discouragement by an APA Division 12 Task Force (2018). Despite these changes across the field, “the Rorschach, TAT, and several other projective techniques remain

among the most frequently used assessment devices in clinical practice” (Lilienfeld et al., 2000, p. 28).

In a monograph on the psychometric reliability of projective techniques relative to their continued use, Lilienfeld et al. warn that scoring procedures for projectives are often unreliable and result in undue subjectivity and individual scorer error (2000). Additionally, normative data tends to be “non-existent or problematic”, leading to a scarcity in demonstrated validity across the majority of tests (Lilienfeld et al., 2000, p. 56). They note that these implications are especially concerning should projective techniques be used in forensic contexts such as “custody disputes, sentencing evaluations, and parole reviews” (Lilienfeld et al., 2000, p. 56). Following a recognition that many mental health professionals will continue to utilize projective tests in their practices in spite of the limitations and controversies, they offer a few targeted notes of professional concern and responsibility. Clinicians (particularly forensic), the authors express, are ethically obligated to disclose the controversial and limited nature of such techniques; certain mental illnesses can be “faked” on the Rorschach and other tests, and the tests are incapable of picking up on such deception; various projective assessments are vulnerable to “situational influences, including subtle verbal reinforcement”; scoring errors are common without a second scorer; and norms are frequently absent, poor, or misleading, further leading to a bias against “North American minority groups and individuals who live outside North America” (2000, p. 57). Clark validates the considerable psychometric concerns within the sphere of projective assessments, simply noting that such issues are “of less concern if projectives are used as informal, hypotheses-generating tools in counseling” (1995, p. 311).

Commonly Used Projective Techniques

As previously noted, an expansive archive of projective assessments have been developed over the past century, and within this collection are a wide range of modalities. To allow for a more thorough understanding of the administration and interpretation of projective assessments, a brief description of some of the most commonly used tests is offered.

Rorschach Inkblot Method

As explained by Hughes et al., during the Rorschach test a practitioner presents a series of 10 cards with ambiguous, chromatic or achromatic inkblots to a single user and asks “What might this be?” about each card (2007, p. 282). The practitioner records the user’s responses verbatim during this “free association” phase, and after all 10 cards have been presented, the practitioner further asks the user “What made it look like that?”, and where on the card the user saw the noted percept (Hughes et al., 2007, p. 282). The user’s responses are then scored on eight dimensions of personality based on both their visual perceptions and the ways they verbalized them (Hughes et al., 2008). The scores for each response are “used to compute various ratios and frequencies, which, in turn, provide information about the individual’s cognitive, emotional, and social functioning” (Hughes et al., 2008, p. 282). The eight dimensions of personality which the Rorschach is believed to gain insight to are: affect, capacity for control and stress tolerance, interpersonal perception and behavior, self-perception, situation-related stress, information processing, cognitive mediation, and ideation, respectively (Hughes et al., 2007).

Thematic Apperception Test (TAT)

As noted, the TAT has been adapted numerous times for a variety of populations. However, the original test consists of 30 images depicting individuals in various ambiguous situations, sometimes alone and sometimes with others, and exhibiting a range of facial

expressions and body language. According to Gibbs et al., users “interpret the scene depicted, identifying the central (main) character, explain what is taking place, describe the thoughts and feelings of the characters, describe the preceding events, and speculate on likely outcomes” (2017, p. 35). The original creators and practitioners today believe that in responding to the images, users project their own “hopes, dreams, fears, frustrations, relationships” onto the images without conscious awareness that they are actually talking about themselves (Gibbs et al., 2017, p. 35).

To score the TAT, Murray’s original method required recording and analyzing every sentence of the user’s story with a focus on apparent motives and feelings of the main character, as well as “press,” or opposing forces in the environment; however, one may quickly realize how tedious and subjective this evaluation system is (Aronow et al., 2001, p. 14). Several scoring techniques have been developed in the years following the original adaptation’s debut in attempts to increase the objectivity of interpretation and the concision of constructs measured (Aronow et al., 2001). While no TAT scoring system has achieved mainstream usage, one worth note is the Main Themes Technique, which was developed by Bellak and Abrams in 1997 (Aronow et al., 2001). This scoring approach instructs test administrators to write three types of summaries based on the user’s story: descriptive, interpretive, and diagnostic (Aronow et al., 2001). The descriptive summary is purely observational, essentially a third-person retelling of the user’s story, whereas the interpretive summary adds a layer of distance by revealing slightly broader themes within the original description (Aronow et al., 2001). The diagnostic summary uses psychodynamic ideas such as conflicts, defenses, and interpersonal patterns to turn the interpretive summary into a definitive statement about the user’s internal experience and worldview (Aronow et al., 2001).

Draw-A-Person

To administer Goodenough's Draw-A-Person (originally "Draw-A-Man") projective test, the practitioner presents the user with a vertical sheet of blank paper and a pencil and requests, "I would like you to draw a picture of a person" (Handler & Thomas, 2014, p. 8). No further instruction is to be given, and any clarifying questions from the user are to be responded to with inconclusive phrases such as "It's up to you; you can do it any way you want" to maintain the ambiguity of the assignment (Handler & Thomas, 2014, p. 8). The practitioner may reassure the user that the technique is not a test of artistic ability; however, should the user draw a stick figure, only a portion of a figure (i.e. only a head) or any other limited representation, the practitioner must provide a new sheet of paper and ask the user to try again and draw a "regular" or "whole" person (Handler & Thomas, 2014, p. 8). When working with adults, following the completion of an initial figure, the user will be asked to draw another person, this time of the opposite sex than the first figure drawn (ex. "You drew a man before; now draw a woman") (Handler & Thomas, 2014, p. 9). Finally, the user is asked to make up a story about the figure(s) they drew to gather more information about the drawing, and the practitioner records this response verbatim (Handler & Thomas, 2014).

The Draw-A-Person (DAP) test can be interpreted through the lenses of a number of constructs, but two primary areas studied since the conception of the assessment include personality and intelligence (Handler & Thomas, 2014). A prime example of a problematic projective technique in terms of its psychometric limitations, scoring systems for the DAP vary widely across practitioners; however, several common features of drawings are believed to indicate larger emotional or behavioral concerns (Maloney & Glasser, 1982). For example, omission of body parts within drawings has been found to be relatively consistent among

“severely regressed” users; transparency, or the visibility of body parts or internal organs through clothing, has been found to correspond significantly with low self-esteem; and distortion, or “body parts drawn out of proportion, parts not connected to the body, and parts drawn in inappropriate areas” has been found to be significantly related to emotional and behavioral disturbances (Maloney & Glasser, 1982, p. 186). More broadly, Langland explains that for clinical uses of the DAP, “basic to interpretation is the assumption that the drawing represents the expression of self, or the body image, in the environment” (1960, p. 105).

Rotter Incomplete Sentences Blank

Differing from the aforementioned methods by focusing on a linguistic approach to projection and offering comparably more structure, sentence completion tests arrived in the projective sphere of psychology in 1897 with Hermann Ebbinghaus, who developed a test as part of a larger inquiry of individual intelligence (Lah, 1989). As with many projective assessments, sentence completion tests have been created and used to evaluate a number of different constructs; the most commonly used example is the Rotter Incomplete Sentences Blank (RISB) (Torstrick et al., 2015). The RISB is expected to assess an individual’s level of adjustment, or their ability to achieve prolonged periods of happiness or euthymia, cope with frustration, begin and maintain focus on tasks, and establish and maintain healthy relationships (Rotter et al., 1992).

The RISB consists of 40 sentence stems of only one or few word(s) (ex. “What annoys me...”) which users complete on their own without additional instruction (Torstrick et al., 2015, p. 496). When scoring the RISB, omissions or fragmented responses are not scored, given that they appear in tests of adjusted users as often unadjusted users, and thus cannot be considered to represent psychological blocks; otherwise, responses are scored on a 7 point scale from 0 to 6,

“with lower scores representing psychological adjustment and higher scores representing higher maladjustment” (Torstrick et al., 2015, p. 496). “Conflict” or madadjustment responses are given a score of 4, 5, or 6 based on the apparent “hostility reactions, pessimism, symptom elicitation, hopelessness and suicidal wishes, statements of unhappy experiences, and indications of past maladjustment” within the created sentences; for example, “I hate...almost everyone” (Rotter & Rafferty, 1950, p. 15). “Positive” or adjusted responses are given a score of 0, 1, or 2 depending on the extent to which they indicate “a healthy or hopeful frame of mind” through “humorous or flippant remarks, optimistic responses, and acceptance reactions” (ex. “The best...is yet to come”) (Rotter & Rafferty, 1950, p. 16). Responses that do not seem to fall into either of these weighted categories, such as those which are considered simply descriptive, or utilize “stereotypes, catch phrases, song titles, and expressions of common cultural cliches”, are labeled “neutral” and given a score of “3” (ex. “Boys...will be boys”) (Rotter & Rafferty, 1950, p. 17). A 2015 study re-examining the construct validity of the RISB found that overall scores successfully discriminated between clinical and nonclinical participants, and high scores were associated with “higher negative affect, more anxiety and depression, irrational thinking, and interpersonal problems” (Torstrick et al., p. 494).

Sandtray Therapy

A projective intervention which further serves as a distinctive form of play therapy, sandtray therapy is an expressive, cross-theoretical therapeutic technique which can be utilized with clients across the developmental lifespan (Sweeney & Homeyer, 2016). It was developed in the early 20th century by Margaret Lowenfeld, who served as a physician and relief worker for students during the Russo-Polish War (Lowenfeld, 1993). Upon her return home to London, she recognized children with the same “expressions, postures, and gestures” as the students she had

witnessed struggling with famine and prison camp conditions (Lowenfeld, 1993, p. 2). Consequently, she was moved to focus on the promotion of children's mental health with consideration of developmental factors such as limitations in verbal expression of emotion, and thus methodized her technique using sand trays and a small collection of toys, insisting that the intervention was "created by the children themselves" (Lowenfeld, 1993, p. 281).

Sandtray therapy is frequently cited as a valuable trauma-informed intervention, given its expressive, nonverbal nature in relation to the neurobiological effects of trauma on the Broca's (speech), limbic (emotional), and frontal cortex (executive functioning) areas of the brain (Sweeney & Homeyer, 2016). Sweeney and Homeyer emphasize the flexibility in potential approaches to sandtray, noting that the specific format of sessions should be adapted by individual clinicians according to their own theoretical orientations, and may further be directive or nondirective (2016). Generally, a sandtray therapy session should begin with the practitioner preparing a collection of miniature toy figures and other objects, and presenting the client with a sand tray filled only with smoothed sand (Sweeney & Homeyer, 2016). For a nondirective approach, the practitioner may instruct the client to select figures or objects that speak to them, and simply create a "world" or "scene" (Sweeney & Homeyer, 2016, p. 44). If intending to tie the session into an ongoing therapeutic topic, the practitioner may instruct the client to create a scene that expresses how they feel related to the theme or concern (Sweeney & Homeyer, 2016).

Sweeney and Homeyer explain that evaluation of a sandtray therapy demonstration will focus not only on the product created by the client, but their process of creation (i.e. hesitation in selecting objects, involvement, planning vs. spontaneous building, etc.) (2016). Further, while the following interpretive discussion should be more reflective than interrogative, the practitioner may query for further understanding of the client's meaning-making process through questions

and directives such as “What would you title this?”, “Tell me about it/what’s happening in the scene”, and “Are you in here?” (Sweeney & Homeyer, 2016, p. 57, p. 60). Comprehensively, sandtray therapy is generally accepted as a projective intervention for the “unfolding and processing of intra- and inter-personal issues” (Sweeney & Homeyer, 2016, p. 6).

Clinical Benefits of Projective Techniques

Thus far, this paper has presented an overview of projectives in counseling and psychology and described several prevailing examples of techniques, while also noting the psychometric limitations of this category of assessments. However, given the aforementioned pervasiveness of their continued use by clinicians despite such deterrents, it seems imperative to equally emphasize the general advantages which preserve this tenuous yet seemingly indestructible reputation.

Enhancement of the Therapeutic Alliance

Unmistakably, projective interventions offer an alternative dynamic to traditional “direct verbal disclosure” in the therapeutic space, while still allowing clients to express themselves (Clark, 1995, p. 311). Even with the acknowledgement that administration of a projective technique may be followed by a discussion of its apparent meaning, clients will experience a break from what may be perceived as the interrogative nature of psychotherapy. While client resistance and its subsequent investigation may have beneficial clinical impacts in the long-term, clients may be more willing and involved in the therapeutic process if they experience an interruption in their self-consciousness or awareness of external clinical judgment. In his classic piece, Clark emphasizes that most clients tend to find projective instruments and techniques interesting; their ambiguity, involvement, and evidently non-threatening presence often lead to a reduction in client defensiveness (1995). This echoes Pepinsky’s seminal finding from decades

earlier, that in using projective techniques the clinician “is able to probe for causes of the client’s problems without arousing resentment or defensiveness” (1947, p. 136). Such perspectives have still been maintained recently, as Wood & Pignatelli introduce an expressive technique with the assertion that distance can be used as an advantage in counseling and supervision (2019). They argue that when individuals are “under-distanced, they are too close and overwhelmed by the emotion of the experience being shared” (Wood & Pignatelli, 2019, p. 233). Through the careful introduction of a third party medium in the therapy space, they note, clinicians may harmlessly access clients’ “inner world and personality functions” to increase awareness of dynamics within the therapeutic alliance (Wood & Pignatelli, 2019, p. 233). Thus, projective techniques assist the clinician in progressing clinical work in a nonintrusive manner while also mitigating ruptures to the therapeutic alliance.

Increasing Client Insight

Through this atypical approach to client information gathering, projective interventions offer a more subtle approach to gaining insight into a client’s internal experience, and further provide a means of comparing unconscious experience to verbal expression of emotion and conflict. An intervention which bypasses conscious thought and encourages clients to project their own deep-rooted beliefs and feelings onto it holds the potential to produce significant breakthroughs during the counseling process. Moller et al. note that projective methods provide “access to unconscious material”, even for clients who either refuse to engage in or experience difficulty in self-reflection (2021, p. 287). Such realizations could be surprising even to the client; perhaps comparable to the concept of advanced empathy, the use of projective techniques may offer a hypothesis about the client’s experience, but originate from an isolated instrument rather than the clinician themselves (Veach, LeRoy, & Bartels, 2003). Clark notes that

contradictions between client projections and typical client disclosures can provide beneficial clinical information, such as a client who verbally expresses positive feelings toward their mother also completing a sentence completion response with “My mother...is a spiteful person” (1995, p. 312). Therefore, projective methods in the counseling process hold the potential to expand both clinician and client insight into deeper therapeutic issues and the client’s awareness thereof.

Hypotheses Development

As warned against by Lilienfeld et al., the results of projective assessments should not be solely or even heavily relied upon to make significant forensic or legal decisions (2000), and thus one would be wise to also abide by this recommendation regarding clinical diagnosis and large-scale decisions about the course of therapy. However, such techniques may be appropriate for use in cases where work with a client seems to have plateaued and would benefit from a change in modality, as well as to generate further hypotheses or hunches about a client’s case or presentation (Clark, 1995). Clark acknowledges that information gleaned from projective assessment is “tentative”, as it can provide “leads or indications about client’s behavior that may later be confirmed or invalidated”; thus, projectives can be appropriately used to formulate ideas about a client’s internal experience and later checked against for accuracy in order to deepen therapeutic work (1995, p. 312). Clark’s assertions support Anastasi’s foundational argument from years prior, that projective work should be used in counseling to suggest insights about the client, but still subjected to considerable further inquiry (1988). More recently, Wood and Pignatelli express the value of using creative techniques in counseling supervision to help trainees conceptualize their cases (2019). If similar practices were used during counseling

sessions themselves, clinicians-in-training could develop similar, and perhaps even more straightforward, hypotheses about clients using information based on projective work.

Treatment Planning

Finally, projective interventions can guide clinicians' responses to certain basic questions regarding client cases, such as whether to continue working with a client, to engage in more extensive assessment, or to refer them to another clinician or resource (Clark, 1995; Drummond, 1992). Once more, apparent insights gained from projective assessment within the therapeutic space are not used solely or definitively to determine next steps, but to generate ideas which inform further decision-making. More specifically, ideas reached through the use of projectives can be used as collateral evidence in establishing goals and objectives for counseling (Clark, 1995), as well used early on in the counseling process to determine primary issues on which to focus (Pepinsky, 1947).

The Practice of Tarot: An Overview

So far, this paper has reviewed the history of projective techniques and presented arguments against their continued use in counseling, as well as evidence for their place in therapy through an expressive rather than a diagnostic or load-bearing role. A litany of examples of projective assessments exist, with only a few commonly used instruments highlighted here. As the definition offered for projective techniques is relatively broad, this paper seeks to present tarot as an appropriate medium for informal projective assessment in the counseling process. In order to provide exposition for this reasoning, a similar overview of tarot is offered through a focused illustration of its history, connections to psychological theory, and existing research on its use in therapy.

Definition and Origins of Tarot

Tarot refers to a deck of 78 cards consisting of 56 minor arcana cards and 22 major arcana cards (Sosteric, 2014). The details of card characteristics vary by deck, but generally a deck's minor arcana will include four suits of swords, pentacles, cups, and staves/wands, with each of these suits containing cards ace through ten, Page/Jack, Knight, King, and Queen. As Sosteric explains, tarot originated in Italy in the 15th century as a secular medium for card gaming, and perhaps gambling (2014). The face cards, or major arcana, of the first decks were directly representative of contemporary royal Italian courts and noble families; further, through cards like the Pope, the Fool, and the Chariot (which can be seen carrying the King), the authority of the church and state was pictorialized (Sosteric, 2014).

Despite tarot's secular beginnings, the practice became associated with cartomancy and divination as a result of a 1781 publication by Antoine Court de Gebelin, a Protestant priest and Freemason (Sosteric, 2014). In an influential essay, he presented a "mystical" interpretation of the Italian tarot of the 15th century, which subsequently caused cartomancers to incorporate tarot into their purportedly prophetic work, and secret societies to co-opt tarot as a component of their "secret knowledge" (Dummett, 1980; Sosteric, 2014, p. 364). This revision of the activities associated with tarot cards has led to the current general association of tarot with the spiritual and the occult. Today, tarot cards are used for divination and cartomancy on a spectrum of practice ranging from tongue-in-cheek novices to experienced individuals practicing modern-day witchcraft and other pagan-leaning forms of spirituality. There is no dominant consensus in interpretation of the power or accuracy of tarot reading, either. Some smirk at the tarot as a party trick game, while others who look toward it with respect and curiosity; some avoid it altogether,

as Sosteric notes that even today “some traditional Christians, in particular those of an evangelical bent, have a powerful belief that the tarot is a book of the devil” (2014, p. 360).

The Practice of Tarot Reading

Just as there is no all-encompassing example of projective technique, there is no singular set of instructions for how to read tarot cards. However, any tarot reading involves the selection of cards by the card reader, or practitioner, and each individual card as well as their placement holds meaning (Watson, 2018). The placement or organization of cards is referred to as a “spread”, and there are several standard spreads which practitioners use and develop upon in their practice (Jorgensen, 2019). Two standard spreads are shortly described here in brief. But before a practitioner even places the cards in a spread, they must shuffle the deck. Typically, the individual who is the focus of the reading, or the querent, will shuffle the deck in order to transfer their spiritual energy into the deck to increase the accuracy of the reading. Some practitioners may give general readings about the querent; if the querent has a specific concern, question, or situation in mind, they may share this with the practitioner and are encouraged to mentally focus on their topic as they shuffle the deck. However, some practitioners may always prefer to shuffle, depending on their style; in this case, the querent may place their hand on the deck before the practitioner shuffles to still allow for the crucial antecedent transfer of energy. If a practitioner is completing a reading for themselves they will shuffle their own deck.

Four-Card Temporal Spread

In one simple temporal spread, the querent divides the shuffled deck into four piles of any preferred size from left to right (see Appendix A). The first three piles may represent either/both the querent’s feelings regarding the concern, question, or situation which is the reading’s focus, or/and the general reality of the situation itself. The pile to the querent’s far left represents the

situation/querent's feelings in the past; the next pile represents the present; the third pile represents the future, if the querent carries on with their same approach to the situation; and the final pile symbolizes the overall lesson the querent must learn. The practitioner reveals the top card of each pile and reveals the meaning of each individual card in the context of past, present, future, and spiritual guidance.

Celtic Cross Spread

The Celtic Cross spread is a popular tarot spread which focuses on a particular question or problem belonging to the querent (Jorgensen, 2019). Each card in this complex, 10-card spread represents a different element or phase within the querent's concern (see Appendix B). While most practitioners tend to hold the same card meanings within the Celtic Cross spread, sometimes the order of cards is shifted; Jorgensen's version of the Celtic Cross is presented here. Card one focuses on the querent's question or situation; card two explains the source of obstacle or struggle; card three represents the querent's goal, or ideal outcome; card four shows past events; card five represents the "merit" of the question; card six depicts present influences on the querent; card seven portrays the querent's attitude toward the situation; card eight depicts sources of strength or power for the querent; card nine reveals the querent's hopes and/or fears about the situation; and card 10 reveals the outcome or prediction of the situation (Jorgensen, 2019, p. 211). As Jorgensen notes and the practitioner may discuss with the querent, this spread operates on the belief that individuals behave based on their mood and motivations, and their actions can further be influenced by elements outside of their control (2019).

Theoretical Orientation in Tarot

At its most basic, tarot reading involves the interpretation of visual symbols with pre-established meanings based on their location in a pattern of cards to make predictions about

one's life. Hundreds of different tarot decks exist and continue to be manufactured, from those most similar to the original decks of the 15th century Italian courts to highly specific sets themed on pop culture and various branches of spirituality. Further, dozens of common card spreads have been used over the years, but many tarot practitioners develop their own methods of card reading. With this broad range of visual imagery and no scarcity of approaches to card reading, tarot easily lends itself as a blank slate upon which tarot practitioners can build their own theory of card interpretation. In the context of counseling, the expressive nature of tarot is particularly apt to be intertwined with various therapeutic theoretical orientations when approaching card reading with the goal of enhancing the counseling relationship and process. The vast scope of theoretical possibility in tarot cannot be understated, but two poignant examples are offered next.

Jungian Tarot

A commonality between many tarot decks is the depiction of archetypal figures within the major arcana, such as the Fool, Death, and the Hierophant, which can all be found in the Rider-Waite deck, one of the most popular tarot decks (Jorgensen, 2019). Famed psychoanalyst Carl Jung argued for the application of an archetypal perspective to tarot, through which therapy clients would ultimately access their unconscious mind's associations of archetypes (Fink, 2022). Broadly, a proposal for a Jungian approach to tarot is based in Jung's theories of synchronicity and symbolism, and further requires inner reflection on the part of the client (Fink, 2022). With regard to synchronicity, Fink explains, a tarot reading exemplifies this meaningful coincidence through the internal experience of the client selecting cards, and the external events in their lives for which they seek answers (2022). Further, the reading relies on the client's individual interpretation of the symbols within each card: for example, Fink offers an example of a querent pulling the Devil card, and initially becoming caught up in its traditional representation of evil

and how it may manifest in her life (2022). A less prophetic and more individually insightful interpretation of this card, Fink offers, was for her to focus on the visual features of the Devil himself and what it could mean for the querent - in this case, the querent's distortion of her boyfriend's actions (2022). Rather than accepting traditional interpretations of tarot cards at face value, Fink notes, the querent gained insight through a highly subjective lens; through symbolic engagement, the cards "open up the multiplicity of meanings that can be applied to a particular situation" (Fink, 2022, p. 64).

Story Weaving (Narrative Therapy)

Story weaving, as Semetsky defines, is a therapeutic technique used to engage with resistant clients in which clinicians designate various "threads" from a client's life, such as their presenting problems, and use them to "weave" together a story and ultimately promote change (2006, p. 189). Semetsky argues that this intervention can be adapted in order to allow a client to weave their own life story, or narrative, by projecting onto the images within a tarot card spread (2006). In this case, rather than telling the client's story, the clinician facilitates the overall process by paraphrasing or communicating the client's story back to them (2006). Semetsky notes that the story a client divulges may disclose their current emotional state, and when projected onto the cards, this story deepens the counseling process through its guidance in alleviating client distress or fixation (2006).

While Semetsky describes story weaving as a stand-alone technique, it bears significant resemblance to Michael White's narrative therapy, a social constructionist approach to counseling which centers on how clients conceptualize their identities through personal stories (Béres, 2022). Narrative therapists interact with clients "from a stance of curiosity", empowering them to describe themselves and their challenges in new, self-compassionate ways (Béres, 2022,

p. 15). Tarot cards provide a medium for clients to practice this narrative reframing, allowing them to project both existing concerns and areas for growth onto the imagery.

Existing Research on Tarot in Counseling

Limited scholarship has been conducted on the use of tarot in the counseling space. Beyond her advocacy for story weaving, Semetsky seems to be the leading researcher and proponent of the therapeutic merit of tarot, and her own studies almost entirely encapsulate the movement's recent developments. While several scholars employ tarot to conceptualize various topics (Beebe, 1985; Watson, 2018), they fail to do so in a clinical context, and thus are not included in this analysis. Thus, Semetsky's most research-based contribution to the issue is exclusively considered here.

In her 2005 paper, she describes 15 case studies in which she engaged in a single clinical tarot reading session, to which each client brought a specific presenting concern (Semetsky). Semetsky utilized a Celtic Cross spread similar to the one previously described here, and each card was interpreted by the reader in the context of their problem based on the meaning of its position in the spread (Semetsky, 2005). Her sample population encompassed a diverse adult sample, and each participant provided feedback on their experience following the session. 13 out of the 15 individuals endorsed a meaningful and productive reading, citing resulting feelings of empowerment and clarity regarding their personal problem, as well as perceived accuracy of the cards in describing the situation and their emotions (Semetsky, 2005).

Semetsky recognizes that this study was limited in scope because she only had a single session with each participant, in effect demonstrating the intake session of the counseling process rather than a thorough realization of a client's presentation and emotional resolution (2005). However, the apparent usefulness of the session according to the majority of participants, she

emphasizes, could be particularly noteworthy within the growing managed care approach to counseling (2005). Thus, while a full course of treatment using tarot as a projective technique may be beneficial, this intervention has positive implications for a short-term model as well.

Interestingly, Semetsky argues that in order to ethically utilize tarot within a clinical context, the clinician must themselves be a seasoned practitioner of tarot reading (2005). This paper disagrees, on account of how tarot may, and perhaps should, be used from a purely projective standpoint inside a counseling relationship. In effect, clinician and client can engage with the cards' symbolism in a similar fashion to other projective techniques, rather than relying on the thorough understanding of traditional card interpretations that a tarot user practicing true cartomancy would. This approach is discussed next in more detail to provide a comprehensive justification for the use of tarot as a projective technique.

A Projective Conceptualization of Tarot

Tarot card reading has been practiced in a divinatory context for over 200 years, and this paper is supportive of the exercise as a component of one's spiritual or personal traditions. However, it argues in contrast to Semetsky's insistence for a clinician's expertise in tarot to use the medium as a projective. In order to ensure that the client is engaging fully in the session rather than seeking direct answers from the clinician or the cards, the use of tarot in counseling should be approached from a purely secular angle. To achieve this, emphasis should be placed on the client's interpretation of card meanings, rather than tarot tradition. Existing card spreads may be particularly useful in different conceptualizations of a client's presenting problem, whether that be considering the elements within the Celtic Cross, a temporal spread, or any number of other highly specific card layouts which function to broaden one's understanding of a

situation. However, to keep the use of tarot secular in session, traditional individual card interpretations should be discarded.

Rather than employing traditional card meanings and weaving them into a client's situation, a client may completely project their own unconscious matter onto the rich symbolic imagery. In effect, tarot cards may be used in a fashion strikingly similar to the Thematic Apperception Test: the clinician may ask prompting questions, and the core of the content is being produced by, or revealed from the unconscious of, the client (Gibbs et al., 2017). The position of a card in the spread can be integrated into the client's understanding through curious inquiry from the clinician. Thus, a counseling session integrating tarot as a projective technique may consist of the client shuffling the tarot cards, organizing them into a specific spread, and considering how each represents their thoughts, feelings, fears, questions, and other facets of their internal experience regarding their presenting concern. Throughout the process, the clinician can ask how the meaning of each card in its position in the spread plays a part, as well as contribute various open-ended questions which allow the client to make disclosures, judgments, and realizations about themselves based on the present cards.

Ethical Considerations for Using Tarot as a Projective Technique

While projection with tarot is proposed here as a relatively open-ended intervention to strengthen the therapeutic relationship and enhance the counseling process, it should still be exercised with caution. Before providing a manual for practice, it is first necessary to consider when this medium is and is not appropriate in a clinical setting, and clients who may and may not be appropriate candidates.

Client Protection

According to the ethical principle of nonmaleficence, a clinician is responsible for considering the potential harm to a client before engaging in a new intervention with them (American Counseling Association, 2014). In the context of potential harm, perhaps the most important factor to consider is a client's level of acuity within their clinical presentation. If a client is experiencing active suicidality and/or psychosis, for example, then limitations should be placed in the therapeutic space to prevent the client from decompensating in their cognitive, emotional, and functional abilities. While using tarot as a projective technique may not seem particularly threatening to one's emotional safety, a clinician should be especially mindful of images and ideas which may be triggering to a client's emotional experience.

Client Presentation

Before engaging in projective tarot with a client, a clinician should confirm that there is no evidence of suicidal or homicidal risk for the client. This is argued because the ambiguous nature of the medium and verbal prompts from the clinician may lead the client to a number of various thought processes, none of which can be limited or structured to remain nonthreatening. To ensure confidence in a client's acuity, a clinician may benefit from performing a suicide risk assessment to gauge a client's level of ideation, intent, and any current or previous suicidal or homicidal behaviors, plans, or gestures. Further, projective assessments such as the Rorschach have traditionally been used to identify mental illnesses such as schizophrenia (Hertz, 1986). Once more, this paper intends only to use this projective technique as an additive rather than a diagnostic feature in the counseling process, but further contends that this method would be best avoided when a client's mental status indicates the presence of delusions, hallucinations, or cognitive impairment which could complicate the projection process. With regard to psychotic

symptoms, it is once again best to avoid open-ended stimuli which could potentially trigger an episode, and when considering clients with cognitive difficulties, some may benefit from more structured interventions.

Client Demographics

Client acuity and mental status aside, some clients may be more agreeable to using tarot as a projective technique and thus may benefit more from the intervention than others. A clinician should directly broach the potential use of the technique, describe the process clearly, and ask for clear consent from the client before engaging in the activity in session. While tarot is presented here from a purely secular approach, it is once again crucial to remember how varied current impressions of and responses to tarot cards may be. Thus, if a client comes from a more traditionally religious or conservative background, and expresses discomfort with using tarot cards in any manner, then this intervention should be avoided.

Further, using tarot as a projective technique may be particularly valuable with clients who have demonstrated interest and engagement with other expressive or creative interventions. If a client has responded positively to drawing exercises, sandtray, movement, or any other nontraditional therapies, then they may be intrigued to approach counseling from a new angle, particularly if sessions feel to have plateaued in terms of therapeutic work. Moreover, if a client has never participated in any sort of expressive counseling practice before but identifies as creative and/or spiritual and enjoys occupying themselves with such activities in their personal life, then they may be amenable to a discussion about using tarot as a projective technique.

Clinician Competence

While a clinician is not required to be a seasoned tarot practitioner in order to use tarot cards for this intervention, it is necessary that they possess a certain level of competence in

counseling and in administering projectives. Due to the ambiguous nature of the task, a clinician must be prepared for the vastness in response that a client may have to projection, and must be able to work with such responses in a manner which is clinically beneficial to the client and to the counseling process. As noted previously, using a projective technique in session can be a unique way to increase client insight and generate hunches about their case (Clark, 1995). Such realizations and ideas, however, may not be encouraging or easily worked through, and thus a clinician should not be using tarot as a projective technique if they do not yet have meaningful professional counseling experience.

Moreover, a clinician would ideally have experience in using and interpreting projective techniques in counseling before attempting to apply tarot in session. While this intervention is not to be used in a diagnostic fashion and thus will not be “scored”, it may still require the clinician to interpret the meaning of the client’s projective responses. During this process, a clinician should make regular use of clarifying, open-ended questions to confirm understanding of the client’s mindset in their projection of their internal experience onto the tarot cards. In general, this paper emphasizes the importance of a clinician’s self-awareness regarding their individual level of clinical competence before embarking on the use of tarot as a projective technique in counseling.

Limited Existing Scholarship

Finally, it must be emphasized that there is a significant paucity of existing research on the use of tarot as a projective technique in counseling. Thus, a clinician interested in attempting the intervention must be prepared for their motives and clinical outcomes to be questioned. In such a case, it is useful to remember that the proposed technique should be utilized only as a way to deepen therapeutic work by stimulating hypotheses, developing the therapeutic alliance, or

disrupting stagnancy, rather than in administering a diagnosis or entirely informing future clinical work. Clinicians are expected to be mindful of their own professional opinions of and experience with projective techniques, as experimenting with any new modality in a clinical setting must be executed with great discretion and care. Following a proposed manual for the application of tarot as a projective technique in counseling, this paper advocates for the continued research of this intervention in order to empirically test its clinical efficiency and benefits. However, until contemporary research contributes more conclusive evidence of tarot's use as a projective technique, clinicians are strongly encouraged to proceed with caution.

Proposed Manual for Applying Tarot as a Projective Technique

Determining Suitability

A clinician's first step in applying tarot as a projective technique in counseling is to determine that the modality suits a particular case. As discussed previously, in deciding if tarot is ethically appropriate to broach with a client, the clinician must consider the client's demographics and presentation to ensure that such a projective technique is unlikely to cause them harm. Any indication from the client that they hold a personal interest in creative, expressive, or even self-reflective activities may increase the likelihood of the client's agreeableness and comfort with the idea of the intervention. Further, there are a number of different situations in which using tarot as a projective may benefit the counseling process: if the clinician and/or client feel that the therapeutic work has reached a plateau or become stagnant, but is incomplete, then such an expressive technique may be useful in approaching the client's presenting concern from another angle. Additionally, some clients may struggle to verbalize their feelings and perspectives about their presenting concerns, or experience difficulty in producing their own reflections without regular prompts from the clinician; using tarot as a projective adds

a medium to inspire such reflections. Finally, many clients enter counseling with the intention of finding answers, or finding a professional to offer advice and tell them exactly how they should resolve their problems. Tarot, on its face, may be particularly appealing to such a client base in that it offers organization to their presenting concerns, and teases prediction of the future, but using it as a projective technique rather than a fortune-telling exercise implores the client to find the answers they need inside their own self-awareness.

Introducing the Intervention

As with any other clinical intervention which relies on client buy-in and active participation, a clinician should comprehensively broach the topic of using tarot as a projective technique before introducing it in session. Depending on the particular case features which make tarot a potentially useful intervention, a clinician may use immediacy to initiate a discussion about continuing the therapeutic work from a fresh, creative approach. Some sample language aiming to capture a client's interest is presented below:

Questions for Introducing a Visual Projective Technique

- It seems like our work together could benefit from approaching this from a new angle. How does that sound to you?
- What comes to mind when I mention the idea of using imagery to process your feelings related to/experience of [presenting problem]?
- Have you ever found that art or other visual imagery has reminded you of [presenting problem]?
- How do you feel about using visual imagery to process/talk more about your feelings/experience of [presenting problem]?

Questions for Introducing Tarot as a Visual Projective Technique

- What previous experiences have you had with tarot?
- What comes to mind when I mention tarot reading, or tarot cards?
- Is tarot reading a part of your own spiritual or personal practice?
- How do you feel about using tarot cards for their imagery, rather than for fortune-telling?

Sample Script for Introducing Tarot as a Visual Projective Technique

It can be helpful to use other tools, even tangible objects, in counseling to expand on our work together. By occasionally changing what our sessions look like by bringing in outside resources, we can make progress and even discover new feelings and new ways of interpreting situations by using objects to inspire us, to help us dig deeper, or help us dig to the problem from a different direction. One way of gaining such insight is to use tarot cards in a slightly different way. Almost as if we were doing a tarot reading together, you may shuffle the deck, then arrange a few cards according to different spreads, in which a certain card position represents a certain point in time, an aspect of a situation, etc. But, instead of looking to traditional tarot card interpretations, we can use the imagery on the cards to create our own meaning. By drawing from your experiences and your responses to looking at the cards in certain positions, we can learn more about how you feel and interpret [presenting problem], and use that to guide our work in future sessions. How does that sound to you?

Applying the Intervention

After a client has consented to using tarot as a projective technique, the clinician may ask the client for their preference in which card spread they would find most useful to process their presenting concern. A clinician should offer as options card spreads with which they are familiar

enough to facilitate without interrupting the process to check for accuracy from a tarot resource. Once a card spread has been determined, the clinician will explain the technique in more detail and check for the client's understanding before initiating the intervention. The following example depicts how a clinician may describe the technique in the context of the client choosing to utilize the four-card temporal spread.

Sample Script for Determining a Card Spread

When using tarot cards as a way to understand feelings about a situation, we can choose to use specific card spreads which have a particular focus. For example, one card spread focuses on time, by giving you a chance to consider how your feelings about [presenting concern] have changed over time, and what we can learn from them. Another card spread helps you to tease apart different parts of [presenting concern], like obstacles you've faced, sources of strength, and other considerations in addition to your feelings and attitude about the situation.

- What kind of focus would be the most helpful for you right now?
- In what area do you need clarity most at this time?
- Which one of those spreads sounds most useful to you right now?

Sample Script for Explaining the Technique

So you've selected the four-card temporal tarot spread. The way this will work is that first, I'll hand you the deck of cards to shuffle however you please. Once you are satisfied with your shuffle, you are going to separate the deck into four separate piles, from your left to your right. Those piles can be as large or as small as you'd like. They can be even or uneven.

Next, we'll go through each card one by one, and you will point out features of the imagery which speak to you. We will consider how your perception of each card reminds of [presenting concern] and your feelings about it. Each card position has a different meaning, so as

you are reflecting on each card's art, we will also consider how it fits into the temporal context - the past, the present, and your possible future. The fourth and final card will represent an overall message about your situation, and we will consider what that may mean to you.

Throughout the process, I will give you space to speak and reflect whatever comes to mind. I will also ask some questions to help you think through the process. How does that sound? Do you have any questions?

Facilitating the Intervention

After the client has confirmed understanding of the process, you may begin as previously described. Hand the deck of tarot cards to the client, and instruct them to shuffle the deck until they are satisfied. Using the example of the four-card temporal spread, once the client has indicated that they have finished shuffling the deck, instruct them to separate the deck into four separate piles in front of them from left to right. Note aloud that the piles do not have to be even. Check if the client is satisfied with their card spread, and then instruct them to reveal the card atop the pile on their far left.

Explain that this card represents their past, in the context of their presenting problem. It could reflect an objective past, and it could also portray their thoughts and/or feelings related to the situation. Encourage them to consider what the card imagery evokes within them. Some prompting questions are offered below:

- What do you think is happening here?
- [If individuals are pictured] What do you think they are feeling? What do you think they are thinking?
 - When have you felt this way because of [presenting concern]?
- How does this relate to your previous experience of [presenting concern]?

- What do you think it means that this card represents your past?

Continue to the present and future card piles, asking similar questions and adapting them according to the relevant temporal setting.

Once you have reached the fourth and final pile, explain to the client that this card represents the overall message or theme of the card reading as related to their presenting concern. Allow them to reveal the card atop the pile and ask prompting questions as needed which place the reading into a larger context:

- What do you think is happening here?
- What do you feel that the overarching message of this card is?
- What does that message mean for you?
 - What does that message mean for [presenting concern]?
- What does this suggest to you about what is working for you? What about what is not working for you?

Post-Intervention Reflection

Clinicians should leave the last five to ten minutes of the clinical hour to debrief with the client about the experience. In facilitating a reflection of the intervention, clinicians may draw from general immediacy or process-based comments to understand how the client is sitting with the session. Some sample reflection questions are offered below.

- What was this experience like for you?
 - Was it easy for you? Hard?
- How did it feel to project your experiences onto the cards?
- How was this activity different from our typical conversations?
 - How was it similar?

- What was it like to work through the cards with me?
- What came up that surprised you?
- Does this feel like a useful medium for you to process issues?
- How are you feeling right now?
- What do you need right now as we end our session?

Concluding Considerations

Projective techniques have both an enduring and contentious history in counseling, and their use continues in counseling rooms today. With respect to legitimate concerns regarding the psychometric and diagnostic capabilities of most projective techniques, clinicians are strongly encouraged to approach projective interventions as a creative complement to traditional psychotherapy, and to avoid relying on their interpretations as symptomatic and definitive. Rather, tarot is presented here as an under-researched application of a projective technique which may strengthen the therapeutic relationship as well as deepen and develop clinical work by building client insight.

Significant attention is given to the ethical implications of undertaking this neoteric intervention, and thus clinicians are encouraged to be judicious in their decision-making about appropriateness of fit for specific clients and clinical cases. The screening process may produce unique challenges; of course a clinician cannot predict exactly how an individual will respond to an intervention even following a thorough discussion beforehand, but great effort should be taken to avoid the risk of a client leaving a session under the impression that their counselor “read their future”. Before adopting tarot as a part of their clinical practice, clinicians should familiarize themselves with the competencies for addressing spirituality and religion in counseling set forth by the Association for Spiritual, Ethical, and Religious Values in Counseling

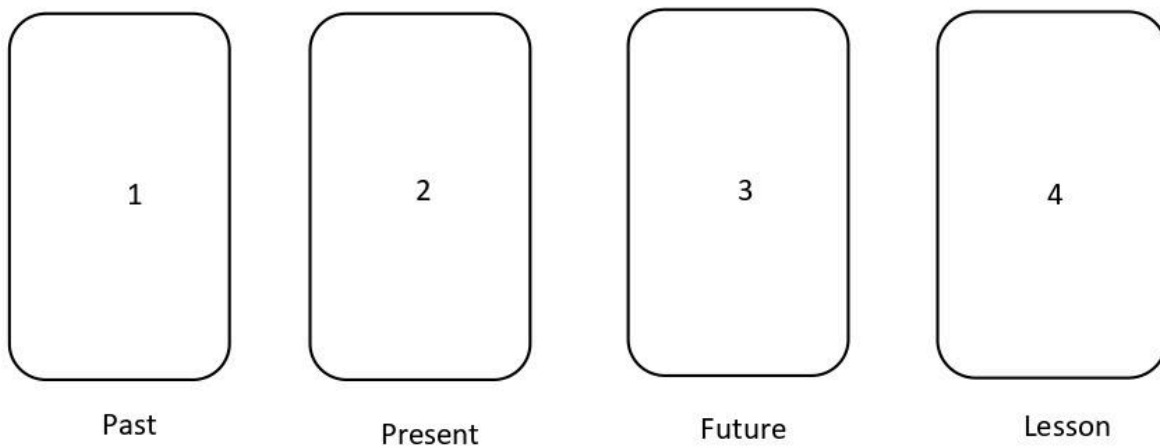
(ASERVIC) division of the American Counseling Association. These 14 guidelines emphasize that counselors are not only mindful, but competent in using spirituality and religion in counseling while adhering to principles such as culture and worldview, counselor self-awareness, and human and spiritual development (Association for Spiritual, Ethical, and Religious Values in Counseling, 2009). Clinicians may refer to these competencies to gauge their preparedness to add this technique to their repertoire and appropriately discuss its meaning and associations with clients. While all of the listed competencies are of relevance, this paper underscores the importance of five in particular: competency two, which reminds clinicians of the influence of a client's spiritual/religious beliefs (or lack thereof) on their worldview and psychosocial functioning; four, which requires that clinicians are aware of their own spiritual/religious beliefs, values, and their impacts on the client and the counseling process; seven, which emphasizes the importance of communication through clinicians' acceptance and sensitivity toward client spiritual/religious beliefs; eight, that clinicians use religious/spiritual concepts of which clients express acceptance; and thirteen, which establishes clinician ability to adjust therapeutic techniques as necessary to incorporate clients' religious/spiritual perspectives (Association for Spiritual, Ethical, and Religious Values in Counseling, 2009).

This paper highlights the potential benefits of this intervention in order to encourage further research and case study. However, given tarot's underrepresentation in academic and clinical scholarship as a projective technique, its precarity as a method cannot be over-emphasized. Further inquiry is recommended to gauge its general effectiveness in advancing the counseling process, as well as its ability to give clients who struggle to verbalize their internal experiences a novel and visually ambiguous medium to spur self-reflection and spark self-understanding. Specifically, future research may investigate if specific client

demographics, beyond the general risk-related rule-outs and individuals with a spiritual or creative streak mentioned in this paper, show particular growth in response to this modality. Moreover, it is unclear if certain clinical or personal concerns and presentations (i.e. anxiety, depression, interpersonal conflict) are benefitted or mitigated by the use of tarot in session. Ultimately, the use of tarot as a projective technique emboldens clients to discern their inner experiences by bringing unconscious or latent emotions and cognitions into conscious awareness through a creative and expressive practice, and its value and stipulations in the counseling process are heartily encouraged to be explored more fully.

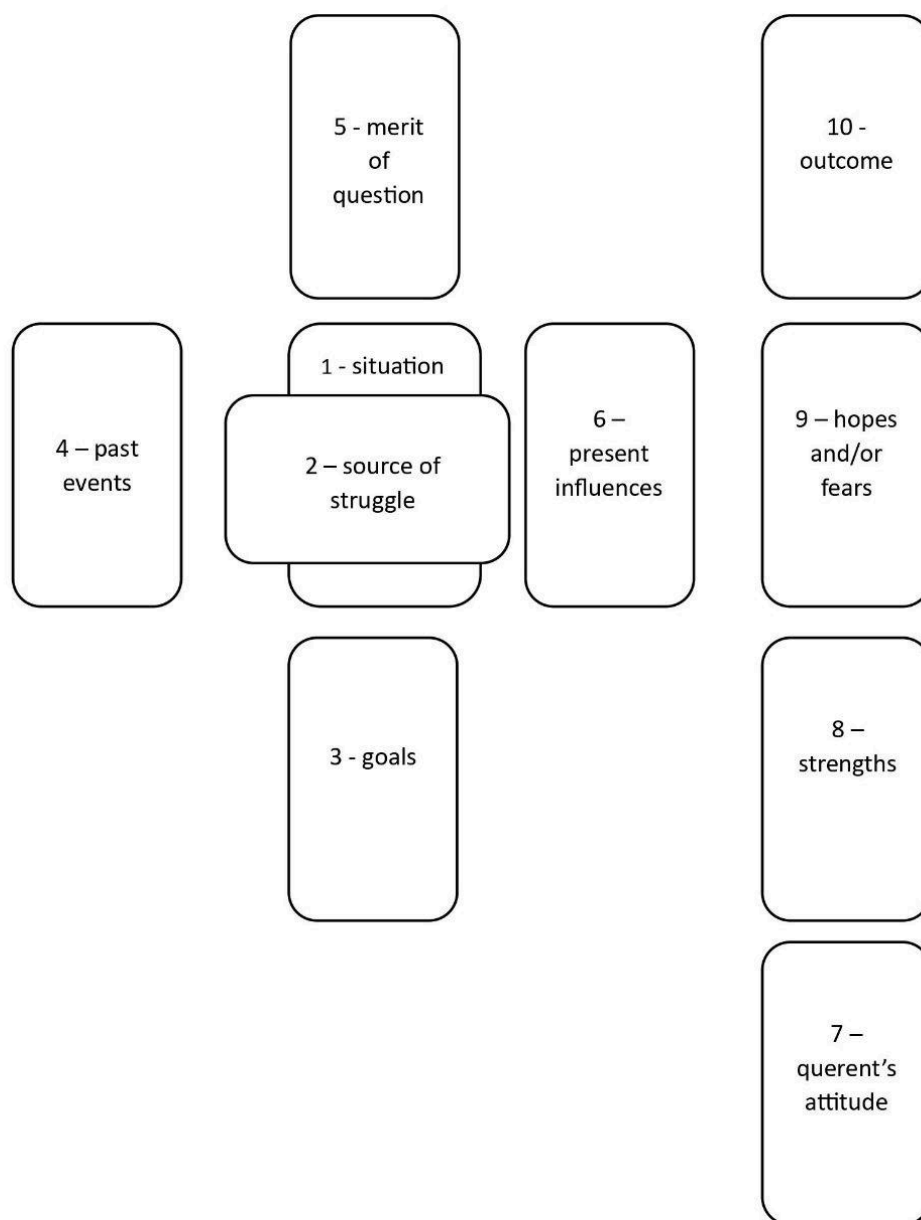
Appendix A

Four-Card Temporal Tarot Spread



Appendix B

Celtic Cross Tarot Spread



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